

Public Comment on HHS Health Sector AI RFI - Request for Information: Accelerating the
Adoption and Use of Artificial Intelligence as Part of Clinical Care

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Executive Summary

This paper announces the Department of Health and Human Services Sector's request for public comments on how it can accelerate the adoption of artificial intelligence within healthcare while upholding ethical considerations and civil liberties. The Department seeks feedback from those buying, building, evaluating, using, and receiving care from AI tools embedded within clinical care systems as well as those who wish to do so but face barriers. The RFI poses ten specific questions addressing barriers to AI adoption, regulatory changes needed, evaluation methods, interoperability challenges, and patient concerns. HHS aims to establish a forward-leaning, industry-supportive approach that reduces uncertainty and ensures AI deployment is used effectively to enhance productivity, reduce costs, and overall improve health outcomes.

Introduction

The U.S. health care system currently stands at a critical crossroad. National health expenditures reached \$4.9 trillion in 2023, with per capita costs at \$14,570, and healthcare now comprises 17.6% of GDP⁴, a figure projected to increase to 20.3% by 2033⁵. These escalating costs place an enormous burden on American families, with 44% of adults reporting difficulty affording healthcare costs and approximately 28% facing problems paying for care in the past year.⁶ Simultaneously, the system faces an extreme workforce crisis, with projections of a shortage exceeding 187,000 full-time equivalent physicians by 2037⁷, and approximately 45% of physicians reporting burnout in 2025.⁸ Amid these mounting pressures of cost and workforce strain, artificial intelligence emerges as a potentially transformative tool in reshaping the problems that hinder the healthcare system today.

Despite AI's promise, its translation into widespread clinical use has been notably limited. Research has identified substantial barriers to adoption across technical, organizational, and human aspects. A systemic review identified 16 key barriers including data quality and bias, infrastructure limitations, financial constraints, workflow misalignment, inadequate training, and potential lack of transparency and accountability.⁹ A 2025 survey of major U.S. health systems found that while ambient clinical documentation tools achieved 100% adoption activity, success rates varied dramatically across use cases, with only 38% reporting high success in AI for clinical risk stratification despite widespread deployment.

⁴ Healthcare Spending in the U.S. 2025 | Stats & Facts. (2025, July 23). The Global Statistics. <https://www.theglobalstatistics.com/united-states-healthcare-spending-statistics/>

⁵ Keehan, S. P., Madison, A. J., Poisal, J. A., Cuckler, G. A., Smith, S. D., Sisko, A. M., Fiore, J. A., & Rennie, K. E. (2025). National Health Expenditure Projections, 2024–33: *Despite Insurance Coverage Declines, Health To Grow As Share Of GDP*. Health Affairs, 44(7). <https://doi.org/10.1377/hlthaff.2025.00545>

⁶ Sparks, G., Lopes, L., Montero, A., Presiado, M., & Hamel, L. (2025, July 11). *Americans' challenges with health care costs*. KFF. <https://www.kff.org/health-costs/americans-challenges-with-health-care-costs/>

⁷ *AAG Healthcare Recruitment and Consulting*. (2025). Aag.health. <https://www.aag.health/post/healthcare-staffing-statistics>

⁸ Mathewes, F. (2026, January 7). *Retirement, burnout fuel physician shortage: 4 new stats*. Becker's ASC. <https://www.beckersasc.com/asc-news/retirement-burnout-fuel-physician-shortage-4-new-stats/>

⁹ Abdelwanis, M., Simsekler, M. C. E., Gabor, A. F., Sleptchenko, A., & Omar, M. (2025). Artificial intelligence adoption challenges from healthcare providers' perspectives: A comprehensive review and future directions. *Safety Science*, 193, 107028. <https://doi.org/10.1016/j.ssci.2025.107028>

There are a multitude of barriers to AI adoption. On the technological front, healthcare organizations struggle with immature AI tools, poor data quality, and incompatibility with existing healthcare infrastructure. Among the concerns of allied healthcare professionals are reduced human contact with patients and perceived reductions in healthcare quality due to the implementation of AI. Organizationally, there is insufficient leadership support and regulatory uncertainty. A 2025 survey found that 70% of hospital leaders experienced at least one AI pilot failure, while 80% found it difficult to verify vendors' AI claims without formal governance.¹⁰ On the human front, clinicians lack adequate training and often resist adoption, fearing replacement or increased workload, which are both compounded by existing staff shortages.

The European Union has adopted a comprehensive risk-based regulatory framework through its AI act, which classifies most healthcare AI solutions as “high risk” and requires strict pre-market testing, validation, and monitoring.¹¹ These international regulatory approaches can act as an instructive comparison for the HHS. The EU AI act implementation began in February 2025 with compliance deadlines extending through July 2025¹², establishing one of the world's most stringent regulatory regimes. In contrast, the United Kingdom has chosen to not pass specific AI laws, but instead encourages regulators to apply existing technology-neutral legislation while adopting a pro-innovation stance.¹³ Australia has similarly taken an approach of prioritizing a national risk-based framework without adopting broader AI legislation.¹⁴

The urgency of potential AI adoption is compounded by the magnitude of healthcare system challenges. Currently, 90% of the \$4.9 trillion in annual healthcare expenditures are for people with

¹⁰ *More hospitals are adopting AI. But do they have a governance strategy?* (2025). Advisory. <https://www.advisory.com/daily-briefing/2025/12/16/ai-governance>

¹¹ Palaniappan, K., Yan, E., & Vogel, S. (2024). Global Regulatory Frameworks for the Use of Artificial Intelligence (AI) in the Healthcare Services Sector. *Healthcare*, 12(5), 562–562. <https://doi.org/10.3390/healthcare12050562>

¹² Team, L. N. (2025, May 21). *EU & UK AI Healthcare Regulation Tracker For 2025*. Legalnodes.com; Legal Nodes. <https://www.legalnodes.com/article/ai-healthcare-regulation>

¹³ Egan, D. (2024). *AI in healthcare: legal and ethical considerations at the new frontier*. Aoshearman.com. <https://www.aoshearman.com/en/insights/ao-shearman-on-life-sciences/ai-in-healthcare-legal-and-ethical-considerations-at-the-new-frontier>

¹⁴ *The EU AI Act's impact on medical devices*. (2025, March 28). PharmaLex. <https://www.pharmalex.com/thought-leadership/blogs/what-global-ai-regulations-mean-for-medical-device-manufacturers/>

chronic and mental health conditions¹⁵, presenting a significant opportunity for AI-driven prevention, early detection, disease management. Hospital expenses grew 5.1% in 2024, outpacing overall inflation of 2.9%, with labor force costs now accounting for 56% of total hospital spending.¹⁶ Additionally, physicians spent an estimated 30-50% of their time on non-clinical tasks such as documentation, coding, and insurance-related activities.

Rather than implementing a comprehensive regulatory structure, HHS proposes using regulation, reimbursement, and research & development to accelerate AI adoption while maintaining public trust. The RFI explicitly acknowledges the “inherent flaws in legacy payment systems” and seeks to “ensure that the potential promises of AI innovations are not diminished through inertia.” This approach represents a bet that market-oriented incentives and strategic R&D investments can achieve faster AI adoption and better health outcomes than Europe’s precautionary regulatory model, while avoiding the innovation-stifling effects of slow reimbursement or regulations that have historically plagued US healthcare technology adoption.

¹⁵ Centers for Disease Control and Prevention. (2024, July 12). *Fast Facts: Health and Economic Costs of Chronic Conditions*. Chronic Disease. <https://www.cdc.gov/chronic-disease/data-research/facts-stats/index.html>

¹⁶ American Hospital Association. (2025, April). *America’s hospitals and health systems continue to face escalating operational costs and economic pressures as they care for patients and communities*. [Www.aha.org](https://www.aha.org/costsofcaring); American Hospital Association. <https://www.aha.org/costsofcaring>

Policy Gaps

Although integrating AI to strengthen healthcare is a promising goal, practical implementation challenges remain significant. When supporting the hopeful long-term viability, key elements identified in the proposed policy must be carefully evaluated.

Elements including broad data collection requirements and AI adoption reliance can burden smaller organizations and local communities. This is true especially for areas with limited resources as this could reduce participation and limit the policy's overall desired impact. Thus, to minimize risk and enhance impact, targeted amendments and improvements can be utilized to improve effectiveness.

When applied, this implementation would demand expanded information collection from a diverse, wide range of stakeholders among healthcare providers, manufacturers, distributors, and specified private sector organizations. The collected data could support planning, allow for early identification of vulnerabilities in the supply chain, and also improve the initiative's alignment with other national preparedness acts. Over time, AI-based tools could enhance the supply chain's speed and accuracy in monitoring. Furthermore, this infrastructure could also support rapid AI deployment in urgent public health emergencies.

Despite these anticipated benefits, potential challenges are still present. First, the scale may impose a disproportionate burden on smaller organizations, communities, and rural partners. Reporting requirements and participation in surveys, interviews, and other methods of engagement may potentially strain limited staffing environments and capacity, reducing participation and creating disproportionate data quality.

Additionally, long-term established reliance on AI systems risks worsening inequalities in adoption systems. Well-resourced institutions can more easily adopt advanced AI than underfunded regions, creating unequal benefits and skewed data representation. Similar disparities emerged following the implementation of the HITECH Act and its Meaningful Use incentives. Researchers observed that rural and smaller healthcare providers adopted electronic health records at

significantly lower rates than larger, urban providers, largely due to resource constraints¹⁷. These historical inequities clearly highlight the need for deliberate measures to ensure equitable AI implementation.

Finally, implementation must be carefully managed to avoid delays and setbacks that could undermine trust among healthcare professionals and between administrators and the public.

Recommendations

To address these risks, the following recommendations should be considered:

- Streamline data collection processes to reduce administrative burden.
- Provide targeted support for smaller and rural healthcare organizations.
- Establish clear regulatory guidelines for AI adoption in clinical settings.
- Implement continuous monitoring and evaluation mechanisms.
- Foster cross-sector collaboration between federal agencies and healthcare providers.

Policy Implications

These proposed recommendations will accelerate the adoption of AI within healthcare, providing both benefits and challenges. By encouraging the use of AI to enhance supply chain resilience, these recommendations will improve the speed and accuracy of healthcare logistics nationwide. Data collected from healthcare providers, manufacturers, distributors, and private organizations could allow for early identification of risks and, in turn, create quicker and more effective responses to public health emergencies. In the long term, this could create a more responsive healthcare system capable of managing large scale crises.

There are several implications which warrant careful consideration. First, the extensive data collection and AI adoption may place a disproportionate burden on small healthcare organizations,

¹⁷ Anzalone, A. J., Geary, C. R., Dai, R., Watanabe-Galloway, S., McClay, J. C., & Campbell, J. R. (2025). Lower electronic health record adoption and interoperability in rural versus urban physician participants: a cross-sectional analysis from the CMS quality payment program. *BMC Health Services Research*, 25(1). <https://doi.org/10.1186/s12913-024-12168-5>

rural communities, and under-resourced providers, who often lack the technical capability for rapid AI adoption, possibly leading to data quality issues. As seen following the HITECH Act's implementation, smaller rural providers may lag behind better-funded providers, further exacerbating the disparity and wealth gap.

To ensure effectiveness and equitable outcomes, the policy should prioritize streamlined data collection processes and focus support on small and rural healthcare organizations. The policy should simultaneously implement clear regulatory guidelines for AI use and consistent steps to evaluate the ethical and operational challenges of rapid AI adoption. Taken together, these improvements can ensure AI integration is effective without widening existing inequalities.

Conclusion

The Department of Health and Human Services' (HHS) initiative to accelerate AI adoption across the healthcare sector reflects an important step in the modernization of the U.S. healthcare system. The integration of AI offers the potential to enhance productivity, improve care delivery, and strengthen the healthcare supply chain. However, the success of this effort will depend on a dual focus on innovation and equity. This policy must take into consideration the steps to ensure that smaller and rural healthcare providers are not excluded from the benefits of AI due to resource constraints or regulatory uncertainty.

To realize AI's promise effectively, HHS must pursue streamlined data collection, targeted technical and financial support to under-resourced institutions, and transparent regulatory guidelines. Furthermore, continuous evaluation and stakeholder engagement will help create and maintain public trust and ensure that AI adoption is aligned with ethics.

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